

Cefuroxim STADA 250mg Film coated caplet

COMPOSITION

Active Ingredient: Each film coated caplet contains cefuroxime axetil equivalent to 250mg of cefuroxime

Other Ingredients: Microcrystalline cellulose, maize starch, crocarmellose sodium, magnesium stearate, talc, hypromellose, polyethylene glycol, titanium dioxide.

PHARMACODYNAMIC PROPERTIES

The in vivo bactericidal activity of cefuroxime axetil is due to cefuroxime's binding to essential target proteins and the resultant inhibition of cell-wall synthesis.

Cefuroxime has bactericidal activity against a wide range of common pathogens, including many beta-lactamase-producing strains. Cefuroxime is stable to many bacterial beta-lactamases, especially plasmid-mediated enzymes that are commonly found in enterobacteriaceae.

Cefuroxime has been demonstrated to be active against most strains of the following microorganisms both in vitro and in clinical infections.

Aerobic Gram-Positive Microorganisms:

Staphylococcus aureus (including beta-lactamase-producing strains), *Streptococcus pneumoniae*, *Streptococcus pyogenes*.

Aerobic Gram-Negative Microorganisms:

Escherichia coli, *Haemophilus influenza* (including beta-lactamase-producing strains), *Haemophilus parainfluenzae*, *Klebsiella pneumoniae*, *Moraxella catarrhalis* (including beta-lactamase-producing strains), *Neisseria gonorrhoeae* (including beta-lactamase-producing strains)

Spirochetes: *Borrelia burgdorferi*

PHARMACOKINETIC PROPERTIES

Absorption and Metabolism: After oral administration, cefuroxime axetil is absorbed from the gastrointestinal tract and rapidly hydrolyzed by nonspecific esterases in the intestinal mucosa and blood to cefuroxime. Absorption of the tablet is greater when taken after food (absolute bioavailability of cefuroxime tablets increases from 37% to 52%). Cefuroxime is not metabolized.

Distribution: Cefuroxime is subsequently distributed throughout the extracellular fluids. The axetil moiety is metabolized to acetaldehyde and acetic acid. Approximately 50% of serum cefuroxime is bound to protein.

Renal Excretion: Cefuroxime is excreted unchanged in the urine; in adults, approximately 50% of the administered dose is recovered in the urine within 12 hours. Because cefuroxime is renally excreted, the serum half-life is prolonged in patients with reduced renal function. Despite the lower elimination of cefuroxime in geriatric patients, dosage adjustment based on age is not necessary.

INDICATIONS

Lower respiratory tract infections: pneumonia, acute bronchitis, and acute exacerbations of chronic bronchitis.

Upper respiratory tract infections: ear, nose and throat infections, such as otitis media, sinusitis, tonsillitis, pharyngitis.

Genito-urinary tract infections: pyelonephritis, cystitis and urethritis.

Skin and soft tissue infections: furunculosis, pyoderma, impetigo.

Gonorrhea, acute uncomplicated gonococcal urethritis, and cervicitis.

DOSAGE AND ADMINISTRATION

Cefuroxime STADA may be administered without regard to meals.

Adults:

Pneumonia: 500 mg bd

Bronchitis: 250 mg bd

Upper resp tract, GUT, skin and soft tissue infection, pyelonephritis: 250 mg bd

UTI: 125mg bd

Uncomplicated gonorrhea: 1,000 mg as single dose

Children

Most infections: 125 mg bd

Otitis media and more severe infections: 250 mg bd.

Children under 5 years: oral suspension advised.

There is no experience in children under 3 months of age.

SPECIAL WARNINGS AND PRECAUTIONS FOR USE

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Cefuroxim STADA, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Cefuroxim STADA must be discontinued immediately and appropriate alternative therapy instituted.

If after administration of cefuroxime axetil sensitivity reactions occur, the use should be discontinued immediately and an appropriate treatment should be established.

As with other broad spectrum antibiotics, prolonged use of cefuroxime axetil may result in the overgrowth of non-susceptible organisms (e.g. candida, clostridium difficile, enterococci) which may require interruption of treatment.

Cefuroxime should be used with caution in the patients with impaired renal function.

In patients who develop severe diarrhea during or after use of cefuroxime axetil, the risk of life threatening pseudomembranous colitis should be taken into account and the use of cefuroxime axetil should be discontinued.

The use of cefuroxime axetil is not recommended in patients with severe gastrointestinal disease, since in these situations a sufficient absorption can not be guaranteed. Administration of a parenteral formulation of cefuroxime should be considered.

PREGNANCY AND LACTATION

Pregnancy: This drug should be used during pregnancy only if clearly needed.

Nursing mothers: Because cefuroxime is excreted in human milk, consideration should be given to discontinuing nursing temporarily during treatment with cefuroxime axetil.

CONTRA-INDICATIONS

Cefuroxime are contraindicated in patients with known allergy to the cephalosporin group of antibiotics or to any of the excipients.

INTERACTION WITH OTHER DRUGS

Simultaneous use of medicines enhancing the pH of the stomach decreases the bioavailability of cefuroxime axetil. It is recommended to avoid this combination.

Since bacteriostatic drug may interfere with the bactericidal action of cephalosporins, it is advisable to avoid giving tetracyclines, macrolides, or chloramphenicol in conjunction with cefuroxime axetil.

Probenecid competes with cefuroxime for renal tubular secretion resulting in higher and more prolonged plasma concentrations of cefuroxime.

The use of cefuroxime axetil may be accompanied by a false positive Coombs test. This may interfere with the performance of cross matching tests with blood. Cefuroxime may interfere with the determination of glucose in urine with copper containing reagentia (Benedict- or Fehling-solution, Clinitest). For the determination of blood-and plasma sugar levels in patients receiving cefuroxime axetil, the glucose-oxidase- or hexokinase method is recommended.

In common with other antibiotics, cefuroxime axetil may affect the gut flora, leading to lower estrogen reabsorption and reduced efficacy of combined oral estrogen/progesterone contraceptives.

Cephalosporin antibiotics at high dosage should be given with caution to patients receiving potent diuretics, aminoglycosides, or amphotericin as these combination increase the risk of nephrotoxicity.

SIDE EFFECT

The following events have been identified in patients treated with cefuroxime tablets and were reported spontaneously.

General: The following hypersensitivity reactions have been reported: anaphylaxis, angioedema, pruritus, rash, serum sickness-like reaction, urticaria.

Gastrointestinal: Pseudomembranous colitis

Hematologic: Hemolytic anemia, leukopenia, pancytopenia, thrombocytopenia, and increased prothrombin time.

Hepatic: Hepatic impairment including hepatitis and cholestasis, jaundice.

Neurologic: Seizure.

Skin: Erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis.

Urologic: Renal dysfunction.

In addition to the adverse reactions listed above that have been observed in patients treated with cefuroxime axetil, the following adverse reactions and altered laboratory tests have been reported for cephalosporin-class antibiotics: toxic nephropathy, aplastic anemia, hemorrhage, increased BUN, increased creatinine, false-positive test for urinary glucose, increased alkaline phosphatase, neutropenia, elevated bilirubin, and agranulocytosis.

OVERDOSE

Overdosage of cephalosporins can cause cerebral irritation leading to convulsions. Serum levels of cefuroxime can be reduced by hemodialysis and peritoneal dialysis.

SHELF LIFE AND STORAGE INSTRUCTIONS

The expiry date of this medicinal products is printed on the box and blister foil. Protect from light. Please store below 30°C. Do not use this medicinal product after the expiry date.

PREPARATIONS AND PACK SIZE

Cefuroxim STADA 250 mg Film coated caplet appears as white to off-white film coated caplet scored on one side and engraved "250" on the other side. They are packed in aluminum/aluminum blister and packed in card box (packs of 10 film coated caplets).

KEEP DRUG OUT OF REACH OF CHILDREN.

CAREFULLY READ THE PACKAGE INSERT BEFORE USE.

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Manufactured by

PYMEPHARCO

(PYMEPHARCO is a subsidiary of STADA

ARZNEIMITTEL AG)

166-170 Nguyen Hue Str, Tuy Hoa City, Phu Yen Province,
Vietnam.